

Patient: **Doe, Jane R.** · MRN: **00831742** · DOB: 1957-03-14 (68F)
· Sex: Female

Subjective

68F seen 8 days post-discharge from Mercy General after admission for acute decompensated HFrEF. Reports improved dyspnea, currently NYHA class II. Down 2 lb since discharge. No chest pain, no syncope. Adherent with all discharge medications.

Problem List

- HFrEF, LVEF 30% (ischemic etiology)
- CAD s/p PCI to LAD with DES (2022)
- Persistent atrial fibrillation, CHA₂DS₂-VASc 5, on apixaban
- Hypertension — currently above goal
- Hyperlipidemia
- Type 2 diabetes mellitus
- Chronic kidney disease, stage 3

Current Medications (verified today)

Lisinopril 20 mg PO daily · Metoprolol succinate 50 mg PO daily · Furosemide 40 mg PO daily · Apixaban 5 mg PO BID · Atorvastatin 40 mg PO daily · Aspirin 81 mg PO daily · Metformin 1000 mg PO BID.

Vitals

BP 138/84 mm Hg · HR 72 (irregular) · Weight 71.5 kg · BMI 27.4.

Labs (this visit)

BMP: Na 139, K 4.1, Cr 1.4, eGFR 45 · NT-proBNP 1,640 pg/mL · LDL-C 95 mg/dL · HbA1c 7.6%.

Assessment and Plan

1. HFrEF (LVEF 30%, ischemic): Patient is on lisinopril and metoprolol succinate, but is NOT on sacubitril/valsartan, an MRA, or an SGLT2 inhibitor. GDMT is incomplete. Discussed transition to sacubitril/valsartan, addition of spironolactone (K and eGFR permit), and starting dapagliflozin. Patient agreeable; will start MRA today and transition ACEi to ARNI after 36-hour washout next week. SGLT2i to be started concurrently.

2. Atrial fibrillation: Persistent, rate-controlled on metoprolol. Continue apixaban 5 mg BID for stroke prevention (CHA₂DS₂-VASc 5).

3. CAD s/p PCI: Continue aspirin 81 mg daily and atorvastatin 40 mg. LDL-C 95 on max-tolerated statin — discussed adding ezetimibe given very high-risk ASCVD status.

4. Hypertension: BP 138/84, above goal of <130/80. Will reassess after ARNI initiation, which often addresses BP simultaneously.

5. T2DM: HbA1c 7.6 on metformin. SGLT2i will provide additional glycemic benefit on top of HF benefit. Coordinate with PCP.

6. CKD3: Stable, eGFR 45. Will monitor with MRA and ARNI titration.

Electronically signed: Sarah Chen, MD — 2026-04-30 11:08 CT